

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Tamsulosin 400mcg MR capsules for the treatment of Benign Prostatic Hyperplasia (BPH)

Patient Group Direction, version 002, issued 11 September 2026. Supersedes Version 001, 1 November 2025.

Summary of NICE / NICE CKS Guidelines for Benign Prostatic Hyperplasia (BPH)

Overview

- **Benign Prostatic Hyperplasia (BPH):** A common condition in older men characterised by enlargement of the prostate gland.
- **Lower Urinary Tract Symptoms (LUTS):** BPH causes LUTS including frequency, urgency, nocturia, poor stream, hesitancy, and incomplete emptying.
- **Epidemiology:** Primarily affects men aged over 50 years, with prevalence increasing with age.

Assessment of BPH

- **IPSS Score:** International Prostate Symptom Score used to quantify symptom severity (mild, moderate, or severe).
- **Digital Rectal Examination (DRE):** Perform if appropriate to assess prostate size and consistency.
- **Prostate-Specific Antigen (PSA):** Consider testing if indicated and after informed discussion with patient about benefits and limitations.
- **Renal Function:** Check baseline renal function (eGFR) before initiating treatment.
- **Urinary Flow Rate:** Non-invasive objective measure of obstruction severity.
- **Post-Void Residual (PVR):** Measure of urine remaining in bladder after micturition; elevated PVR suggests retention risk.

Management of BPH

- **Watchful Waiting:** Initial conservative approach for mild symptoms; includes lifestyle modifications and regular monitoring.
- **Alpha-blockers:** First-line pharmacotherapy for symptomatic relief of LUTS in BPH; tamsulosin is a selective alpha-1a adrenoceptor antagonist.
- **5-Alpha Reductase Inhibitors:** Indicated for men with large prostate glands; slow disease progression and reduce prostate volume over time.

Red Flags Requiring Specialist Referral

- **Acute Urinary Retention:** Inability to void spontaneously; immediate specialist assessment required.
- **Recurrent Urinary Tract Infections:** May indicate incomplete bladder emptying or other complications.
- **Haematuria:** Blood in urine requires investigation to exclude other pathology.
- **Renal Impairment:** Evidence of renal dysfunction secondary to urinary obstruction.
- **Elevated PSA:** Concern for malignancy requires specialist evaluation.

Patient Group Direction

for the supply/administration of

Tamsulosin 400mcg MR capsules

for the treatment of

Benign Prostatic Hyperplasia (BPH)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Symptomatic relief of lower urinary tract symptoms (LUTS) associated with benign prostatic hyperplasia (BPH) in adult men
Inclusion criteria	Male patients aged 18 years and above Clinical symptoms consistent with BPH (frequency, urgency, nocturia,

	poor urinary stream, hesitancy, incomplete bladder emptying) IPSS (International Prostate Symptom Score) of moderate severity or above (≥ 8) Informed consent obtained after discussion of benefits, risks, and alternatives
Exclusion criteria	Known hypersensitivity to tamsulosin or any excipients in the formulation History of orthostatic hypotension (blood pressure drop on standing) Severe hepatic impairment (Child-Pugh Grade C) Concurrent use of other alpha-1 adrenoceptor antagonists Planned cataract or glaucoma surgery (due to risk of intraoperative floppy iris syndrome - IFIS) Uncontrolled hypertension Acute urinary retention requiring catheterisation or in-patient management Visible or non-visible haematuria (refer: urological investigation before any treatment of symptoms) Current or recurrent urinary tract infection, or dysuria with fever (refer) Palpable bladder, or symptoms suggesting chronic retention: overflow incontinence, a constant feeling of incomplete emptying with a poor stream (refer for post-void residual measurement) Known or suspected prostate cancer, an abnormal digital rectal examination, or a raised PSA (refer) Neurological disease affecting bladder function: multiple sclerosis, Parkinson's disease, spinal cord disease, diabetic neuropathy (refer) Aged under 45: lower urinary tract symptoms at this age are unlikely to be BPH; refer to the GP for diagnosis Symptoms not previously assessed by a GP or urologist, unless the GP is informed on the day of supply and the patient agrees to attend the GP within 6 weeks for examination and, where indicated, PSA testing
Cautions	Orthostatic hypotension: especially in elderly patients or those on concurrent antihypertensive medication; patients should be advised to sit or lie down if dizziness occurs Renal impairment: use with caution in patients with $\text{eGFR} < 10 \text{ mL/min/1.73m}^2$ Hepatic impairment: avoid in severe hepatic disease; use with caution in mild to moderate impairment Concurrent use of PDE5 inhibitors (sildenafil, tadalafil): additive hypotensive effects; monitor blood pressure carefully History of syncope or fainting: increased risk with tamsulosin; consider discontinuation if syncope occurs

	Surgery: inform all healthcare professionals, including ophthalmologists, of tamsulosin use before any planned procedures
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Tamsulosin 400 micrograms modified-release capsules
Legal category	POM (Prescription Only Medicine)
Storage	Store below 25°C. Protect from light and moisture. Keep in original container.
Route/method of administration	Oral. Modified-release capsule to be swallowed whole with water after food (preferably after breakfast). Not to be crushed, chewed, or opened.
Dose and frequency	400 micrograms once daily Take after food, preferably with breakfast Modified-release capsule must be swallowed whole - do not crush, chew, or open the capsule
Quantity to be administered and/or supplied	Up to 28 capsules per prescription (28-day supply)
Maximum or minimum treatment period	Maximum under this PGD: an initial supply of 4 weeks, then, where the IPSS has improved by 3 points or more at the 4 to 6 week review and the patient has been examined by the GP, further supplies of up to 12 weeks each to a maximum of 12 months' continuous treatment, after which the GP takes over prescribing. No improvement at 4 to 6 weeks, or any new exclusion, ends supply under this PGD and the patient is referred.
Adverse effects	Very Common (≥ 1 in 10): Abnormal ejaculation (predominantly retrograde ejaculation) Common (1 in 100 to 1 in 10): Dizziness, headache, rhinitis (nasal symptoms), palpitations, anxiety Uncommon (1 in 1,000 to 1 in 100): Postural/orthostatic hypotension, syncope (fainting), nausea, constipation, diarrhoea, asthenia (weakness), rash, pruritus (itching) Rare (< 1 in 1,000): Priapism (prolonged erection > 4 hours), intraoperative floppy iris syndrome (IFIS) during cataract/glaucoma surgery, Stevens-Johnson syndrome, angioedema

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication and discuss key safety points verbally.
Follow-up advice to be given to patient or carer	Take the capsule after food, preferably with breakfast, to minimise side effects Stand up slowly from sitting or lying down, especially at the start of treatment or after a dose increase, as dizziness may occur Inform any healthcare professional (including eye surgeons and dentists) that you are taking tamsulosin before any planned procedures, especially eye surgery Report any episodes of dizziness, fainting, or lightheadedness to your doctor immediately - these may require dose adjustment or discontinuation Inform your GP or pharmacist if you experience abnormal ejaculation or other sexual dysfunction during treatment Do not crush, chew, or open the capsule - swallow whole to maintain the modified-release formulation Report any signs of priapism (erection lasting longer than 4 hours) immediately to A&E or your GP Seek urgent medical attention if you experience rapid heartbeat, chest pain, or severe dizziness

Valid from date	Expiry date		
11/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Exclusions added: haematuria, current or recurrent UTI, chronic retention, known or suspected prostate cancer, neurological bladder disease, age under 45, and symptoms never assessed by a GP without a GP appointment within 6 weeks. The previous version's summary named these as red flags but the exclusion criteria did not.

A maximum treatment period is stated: 4-week initial supply, continuation only after GP examination and IPSS improvement, 12 months' maximum before GP prescribing. The previous version said 'ongoing treatment'.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v002, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.